

300 mm

8 mm

6 mm

8 mm

For the use of a Registered Medical Practitioner or a Hospital or a Laboratory

MOXIFLOXACIN 0.5% w/v OPHTHALMIC SOLUTION USP
MOVELOX

Moxifloxacin 0.5% w/v Ophthalmic Solution USP
MOVELOX

COMPOSITION

Each mL contains:
Moxifloxacin Hydrochloride USP 5.45mg
equivalent to Moxifloxacin 5mg
Sterile aqueous Vehicle q. s.

Dosage Form

Ophthalmic Solution

Product Description - A yellow coloured, clear solution, free from visible particles of foreign matter.

PHARMACOLOGY:

Pharmacodynamic
Pharmacotherapeutic group: Ophthalmologicals; anti-infectives, other anti-infectives.
ATC Code: S01AE17

Mode of Action:

Moxifloxacin, a fourth-generation fluoroquinolone, inhibits the DNA gyrase and topoisomerase IV required for bacterial DNA replication, repair, and recombination.

Resistance:

Resistance to fluoroquinolones, including moxifloxacin generally occurs by chromosomal mutations in genes encoding DNA gyrase and topoisomerase IV. In Gram-negative bacteria, moxifloxacin resistance can be due to mutations in mar (multiple antibiotic resistance) and the qnr (quinolone resistance) gene systems. Resistance is also associated with expression of bacteria efflux proteins and inactivating enzymes. Cross-resistance with beta-lactams, macrolides and aminoglycosides is not expected due to differences in mode of action.

Susceptibility Testing/Resistances:

There are no pharmacological data correlated with clinical outcome for moxifloxacin administered as a topical agent. As a result, the European Committee on Antimicrobial Susceptibility Testing (EUCAST) suggests the following epidemiological cut-off values (ECOFF mg/l) derived from MIC distribution curves to indicate susceptibility to topical moxifloxacin:

Corynebacterium	ND
Staphylococcus aureus	0.25 mg/l
Staphylococcus coag-neg.	0.25 mg/l
Streptococcus pneumoniae	0.5 mg/l
Streptococcus pyogenes	0.5 mg/l
Streptococcus viridans group	0.5 mg/l
Enterobacter spp.	0.25 mg/l
Haemophilus influenzae	0.125 mg/l
Haemophilus spp.	0.25 mg/l
Moraxella catarrhalis	0.25 mg/l
Morganella morganii	0.25 mg/l
Neisseria gonorrhoeae	0.032 mg/l
Pseudomonas aeruginosa	4 mg/l
Serratia marcescens	1 mg/l

The prevalence of acquired resistance may vary geographically and with time for selected species and local information on resistance is desirable, particularly when treating severe infections. As necessary, expert advice should be sought when the local prevalence of resistance is such that the utility of moxifloxacin in at least some types of infections is questionable.

COMMONLY SUSCEPTIBLE SPECIES

Aerobic Gram-positive micro-organisms:

Corynebacterium species including
Corynebacterium diphtheriae
Staphylococcus aureus (methicillin susceptible)
Streptococcus pneumoniae
Streptococcus pyogenes

Aerobic Gram-negative micro-organisms:

Enterobacteriaceae
Haemophilus influenzae
Haemophilus spp.
Moraxella catarrhalis
Serratia marcescens

Anaerobic micro-organisms:

Progonobacterium acetii

Other micro-organisms:

Chlamydia trachomatis

SPECIES FOR WHICH ACQUIRED RESISTANCE MAY BE A PROBLEM

Aerobic Gram-positive micro-organisms:

Staphylococcus aureus (methicillin resistant)
Staphylococcus coagulase-negative species (methicillin resistant)

Aerobic Gram-negative micro-organisms:

Neisseria gonorrhoeae

Other micro-organisms:

None

INHERENTLY RESISTANT ORGANISMS

Aerobic Gram-negative micro-organisms:

Pseudomonas aeruginosa

Other micro-organisms:

None

Pharmacokinetics

The mean steady state C_{max} and AUC of topical ocular Moxifloxacin is reported to be 2.7ng/ml and 45ng.hr/ml

respectively when dosed 3 times a day bilaterally. The plasma half life of Moxifloxacin estimated to be 13 hrs.

Indication

MOVELOX is indicated for the treatment of patients 1 year of age and older with bacterial conjunctivitis caused by susceptible strains of the following organisms:
Gram-positive bacteria:
Corynebacterium species*

Microbacterium species Micrococcus luteus* (including erythromycin, gentamicin, tetracycline, and/or trimethoprim resistant strains)***
Staphylococcus aureus (including methicillin, erythromycin, gentamicin, ofloxacin, tetracycline and/or trimethoprim resistant strains)

Staphylococcus epidermidis (including methicillin, erythromycin, gentamicin, ofloxacin, tetracycline and/or trimethoprim resistant strains)

Staphylococcus hominis (including methicillin, erythromycin, gentamicin, ofloxacin, tetracycline and/or trimethoprim resistant strains)

Staphylococcus warneri* (including erythromycin resistant strains)

Streptococcus mitis* (including penicillin, erythromycin, tetracycline and/or trimethoprim resistant strains)

Streptococcus pneumoniae (including penicillin, erythromycin, gentamicin, tetracycline and/or trimethoprim resistant strains)

Streptococcus viridans (including penicillin, erythromycin, tetracycline and/or trimethoprim resistant strains)

Gram-negative bacteria:
Aerobacter species Haemophilus "alcalase" (including ampicillin resistant strains)

Haemophilus influenzae (including ampicillin resistant strains)

Haemophilus influenzae (including ampicillin resistant strains)

Moraxella catarrhalis*

Pseudomonas aeruginosa*

Other microorganisms:
Chlamydia trachomatis

* Efficacy for this organism was studied in fewer than 10 infections.

Preoperative and postoperative sterilization (when prophylactic antibiotic treatment is required).

CONTRAINDICATIONS

Moxifloxacin ophthalmic solution is contraindicated in patients with a history of hypersensitivity to moxifloxacin, to other quinolones, or to any of the components in this medication.

WARNINGS AND PRECAUTIONS

NOT FOR INJECTION.

In patients receiving systemically administered quinolones, serious and occasionally fatal hypersensitivity (anaphylactic) reactions have been reported, some following the first dose.

Some reactions were accompanied by cardiovascular collapse, loss of consciousness, angioedema (including laryngeal, pharyngeal or facial oedema), airway obstruction, dyspnoea, urticaria, and itching.

If an allergic reaction to MOVELOX occurs, discontinue use of the medicinal product. Serious acute hypersensitivity reactions to moxifloxacin or any other product ingredient may require immediate emergency treatment. Oxygen and airway management should be administered where clinically indicated.

As with other anti-infectives, prolonged use may result in overgrowth of non-susceptible organisms, including fungi. If superinfection occurs, discontinue use and institute alternative therapy.

Tendon inflammation and rupture may occur with systemic fluoroquinolone therapy including moxifloxacin, particularly in older patients and those treated concurrently with corticosteroids. Following ophthalmic administration of MOVELOX plasma concentrations of moxifloxacin are much lower than after therapeutic oral doses of moxifloxacin, however, caution should be exercised and treatment with MOVELOX should be discontinued at the first sign of tendon inflammation.

Data are very limited to establish efficacy and safety of MOVELOX in the treatment of conjunctivitis in neonates. Therefore use of this medicinal product to treat conjunctivitis in neonates is not recommended.

MOVELOX should not be used for the prophylaxis or empiric treatment of gonococcal conjunctivitis, including gonococcal ophthalmia neonatorum, because of the prevalence of fluoroquinolone-resistant Neisseria gonorrhoeae. Patients with eye infections caused by Neisseria gonorrhoeae should receive appropriate systemic treatment.

The medicinal product is not recommended for the treatment of Chlamydia trachomatis in patients less than 2 years of age as it has not been evaluated in such patients. Patients older than 2 years of age with eye infections caused by Chlamydia trachomatis should receive appropriate systemic treatment.

Neonates with ophthalmia neonatorum should receive appropriate treatment for their condition, e.g. systemic treatment in cases caused by Chlamydia trachomatis or Neisseria gonorrhoeae.

Patients should be advised not to wear contact lenses if they have signs and symptoms of a bacterial ocular infection.

Exacerbation of myasthenia gravis: Fluoroquinolones have neuromuscular blocking activity and may exacerbate muscle weakness in person with myasthenia gravis. Post marketing serious adverse events, including deaths and requirement for ventilator support have been associated with fluoroquinolones use in persons with myasthenia gravis. Avoid fluoroquinolones in patients with known history of myasthenia gravis.

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300 mm

PATIENT INFORMATION

Avoid contaminating the applicator tip with material from the eye, fingers or other source. Systemically administered quinolones including moxifloxacin have been associated with hypersensitivity reactions, even following a single dose. Discontinue use immediately and contact your physician at the first sign of a rash or allergic reaction.

Recommended Dose:

Instil one drop in the affected eye 3 times a day for 7 days. For preoperative and postoperative sterilization: Usually, instil one drop in the affected eye(s) 5 times per day before operation, and 3 times per day after operation.

Route of Administration - Ophthalmic

Overdose and Treatment

No case of overdose has been reported. A topical overdose may be flushed from the eye(s) with warm tap water.

Drug Interactions

Drug-drug interaction studies have not been conducted with MOVELOX solution.

In vitro studies indicate that moxifloxacin does not inhibit CYP3A4, CYP2D6, CYP2C9, CYP2C19, or CYP1A2 indicating that moxifloxacin is unlikely to alter the pharmacokinetics of drugs metabolized by this cytochrome P450 isozymes.

Effects on ability to drive and use machines:

MOVELOX has no or negligible influence on the ability to drive and use machines, however, as with any eye drops, temporary blurred vision or other visual disturbances may affect the ability to drive or use machines. If blurred vision occurs at instillation, the patient should wait until their vision clears before driving or using machinery.

Pregnancy:

There are no adequate data from the use of MOVELOX in pregnant women. However, no effects on pregnancy are anticipated since the systemic exposure to moxifloxacin is negligible. The medicinal product can be used during pregnancy.

Lactation

It is unknown whether moxifloxacin/metabolites are excreted in human milk. Animal studies have shown excretion of low levels in breast milk after oral administration of moxifloxacin. However, at therapeutic doses of MOVELOX no effects on the suckling child are anticipated. The medicinal product can be used during breast-feeding.

Adverse Reactions/Side Effects

System Organ Classification	Frequency	Adverse reactions
Blood and lymphatic system disorders	Rare	haemoglobin decreased
Immune system disorders	Not known	Hypersensitivity
Nervous system disorders	Uncommon Rare Not known	headache paresthesia dizziness
Eye disorders	Common Uncommon Rare Not known	eye pain, eye irritation punctate keratitis, dry eye, conjunctival haemorrhage, ocular hyperemia, eye pruritus, eyelid oedema, ocular discomfort, corneal epithelium defect, corneal disorder, conjunctivitis, blepharitis, eye swelling, conjunctival oedema, vision blurred, visual acuity reduced, asthenopia, erythema of eyelid endophthalmitis, ulcerative keratitis, corneal erosion, corneal abrasion, intraocular pressure increased, corneal opacity, corneal infiltrates, corneal deposits, eye allergy, keratitis, corneal oedema, photophobia, eyelid oedema, lacrimation increased, eye discharge, foreign body sensation in eyes
Cardiac disorders	Not known	palpitations
Respiratory, thoracic and mediastinal disorders	Rare Not known	nasal discomfort, pharyngolaryngeal pain, sensation of foreign body (throat) dyspnoea
Gastrointestinal disorders	Uncommon Rare Not known	diysaesia vomiting nausea
Hepatobiliary disorders	Rare	alanine aminotransferase increased, gamma-glutamyltransferase increased
Skin and subcutaneous tissue disorders	Not known	erythema, rash, pruritus, urticaria

Description of selected adverse reactions

Serious and occasionally fatal hypersensitivity (anaphylactic) reactions, some following first dose, have been reported in patients receiving systemic quinolone therapy. Some reactions were accompanied by cardiovascular collapse, loss of consciousness, angioedema (including laryngeal, pharyngeal or facial oedema), airway obstruction, dyspnoea, urticaria and itching. Ruptures of the shoulder, hand, Achilles, or other tendons that required surgical repair or resulted in prolonged disability have been reported in patients receiving systemic fluoroquinolones. Studies and post marketing experience with systemic quinolones indicate that a risk of these ruptures may be increased in patients receiving corticosteroids, especially geriatric patients and in tendons under high stress, including Achilles tendon.

Paediatric population

In clinical trials, MOVELOX has shown to be safe in paediatric patients, including neonates. In patients under 18 years old, the two most frequent adverse reactions were eye irritation and eye pain, both occurring at an incidence rate of 0.9%.

Based on data from clinical trials involving paediatric patients, including neonates, the type and severity of adverse reactions in the paediatric population are similar to those in adults.

Exacerbation of myasthenia gravis: Post marketing serious adverse events, including deaths and requirement for ventilator support have been associated with fluoroquinolones use in persons with myasthenia gravis.

SELF-LIFE

36 months.

Use the Solution within 28 days after opening the bottle.

STORAGE CONDITION

Before opening - Store below 30°C. Protect from Light.

After opening - Store below 30°C.

PACKAGING INFORMATION

MOVELOX Ophthalmic Solution: Bottle of 5ml packed in a carton with insert.

Manufactured by:

INDOCO REMEDIES LTD.

L-32, 33, 34, Verna Industrial Area, Verna, Goa - 403 722, INDIA.

Regd. Office: 166, C.S.T. Road, Mumbai 400 096, INDIA.

Name & address of Product Registration Holder:

Arms Pharma Sdn. Bhd., 22-1, Jalan PJU 5/20D,

The Strand, 47810 Kota Damansara, Selangor, Malaysia.

Date of Revision – March 2018